

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of COVID-19 vaccines (Comirnaty, Spikevax and
Nuvaxovid)
for protection against COVID-19, 2026/27 season

Guideline Summary: COVID-19 vaccination, 2026/27

The season and eligibility

- The cohorts eligible for NHS vaccination in England in autumn 2026 are adults aged 75 years and over, residents in care homes for older adults, and individuals aged 6 months and over who are immunosuppressed as defined in the COVID-19 chapter of the Green Book.
- Eligibility for autumn 2026 is unchanged from the spring 2025, autumn 2025 and spring 2026 campaigns.
- This PGD covers privately funded vaccination, whether or not the individual is eligible under the NHS programme. It does not replace the national programme, and eligible individuals should be told they can be vaccinated free of charge on the NHS.

Vaccine composition

- Vaccines are updated periodically to match circulating variants. For the 2026 to 2027 season, regulators have recommended a monovalent JN.1 lineage XFG formulation.
- The formulation supplied must be confirmed against the current SPC and pack before administration, as the approved strain may be updated during the season.

Interval and recent infection

- A minimum interval of 3 months is recommended between COVID-19 vaccine doses.
- Following confirmed COVID-19 infection, defer vaccination until the individual has recovered. A 4 week interval from a positive test or symptom onset is commonly applied, and 12 weeks in children and young people aged 5 to 17 who are not in a risk group.
- COVID-19 vaccine may be co-administered with influenza vaccine at a separate site.

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Comirnaty, Spikevax and Nuvaxovid

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Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed immunisation training in line with the National Minimum Standards and Core Curriculum for Immunisation Training, updated for the current season • All users must be competent in the recognition and management of anaphylaxis, and in vaccine handling and cold chain management • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Clinical condition or situation to which this PGD applies

PGD Indication	Active immunisation of individuals aged 12 years and over for the prevention of COVID-19 caused by SARS-CoV-2, using the current seasonal formulation of Comirnaty, Spikevax or Nuvaxovid, in accordance with the current Summary of Product Characteristics and the COVID-19 chapter of Immunisation Against Infectious Disease (the Green Book). Comirnaty is the vaccine of choice under this PGD, with Spikevax and Nuvaxovid available where Comirnaty stock is unavailable. This PGD covers privately funded vaccination, whether or not the individual is eligible under the NHS programme.
Inclusion criteria	• Individuals aged 12 years and over. The vaccine administered must be licensed for the age of the individual; follow the SPC for the product and presentation supplied. • Requires COVID-19 vaccination and either does not qualify for NHS vaccination, or qualifies but prefers to be vaccinated privately. • At least 3 months since the last COVID-19 vaccine dose. • Valid informed consent obtained, from a person with parental responsibility where the individual is under 16 and not Gillick competent.

	• No contraindication to the vaccine as set out below.
Exclusion criteria	• Aged under 12 years. Refer to the GP or a service commissioned to vaccinate children, as age-specific presentations and dose volumes are required. • Known hypersensitivity to the active substance or to any excipient listed in the current SPC, including polyethylene glycol (PEG) for the mRNA vaccines and polysorbate 80 for Nuvaxovid. • History of anaphylaxis to a previous dose of the same vaccine or to any of its components. • History of myocarditis or pericarditis following a previous dose of an mRNA COVID-19 vaccine. Refer for specialist advice; do not give a further mRNA dose under this PGD. • Acute severe febrile illness. Postpone until recovered. A minor infection without fever is not a contraindication. • Confirmed current COVID-19 infection. Defer until recovered. • Less than 3 months since the last COVID-19 vaccine dose, unless a shorter interval is specifically advised in national guidance for that individual. • Bleeding disorder where intramuscular injection has not been assessed as safe by a clinician familiar with the individual's bleeding risk. • Valid informed consent not obtained, or consent from a person with parental responsibility not available where required.
Cautions including any relevant action to be taken	• Facilities and trained staff for the management of anaphylaxis must be available, with immediate access to adrenaline (epinephrine) 1 in 1,000 injection and a telephone. • Observe the individual for 15 minutes after vaccination where there is a history of allergy or previous vaccine reaction. Syncope can occur, particularly in adolescents; vaccinate seated and ensure procedures are in place to avoid injury from faints. • Anxiety related reactions, including vasovagal reactions (syncope), hyperventilation or stress related reactions (for example dizziness, palpitations, increases in heart rate, alterations in blood pressure, paraesthesia, hypoaesthesia and sweating) may occur in association with the vaccination process itself. Stress related reactions are temporary and resolve on their own. Individuals should be advised to bring symptoms to the attention of the vaccination provider for evaluation. It is important that precautions are in place to avoid injury from fainting. • Myocarditis and pericarditis have been reported very rarely after mRNA vaccines, most often in younger males after the second dose. Advise the individual to seek urgent medical attention if they develop chest pain, shortness of breath, palpitations or a fluttering heartbeat after vaccination. • A few cases of capillary leak syndrome (CLS) flare-ups have been reported in the first days after vaccination with Spikevax. Be aware of the signs and symptoms of CLS, primarily hypotension and oedema, so that it is recognised promptly and immediate medical help sought. In an individual with a history of CLS, vaccination should be planned in collaboration with appropriate medical experts. • Pregnancy and breastfeeding: COVID-19 vaccination is recommended in pregnancy for those in an eligible group and is safe while breastfeeding. Where the individual is pregnant,

	confirm the vaccine and indication against current national guidance before proceeding. - Immunosuppression: the immune response may be reduced. Vaccination is still recommended and the individual should be advised that protection may be limited. Individuals who are immunosuppressed are eligible under the NHS programme and should be advised of this. - Anticoagulation and bleeding disorders: individuals on stable anticoagulation may be vaccinated intramuscularly with a 23 gauge or finer needle, with firm pressure applied without rubbing for at least 2 minutes. Advise on the risk of haematoma. - Where an individual is eligible for NHS vaccination, they must be told that they can receive the vaccine free of charge on the NHS before proceeding with a private supply.
Actions if patient is excluded or declines treatment	Advise on alternative options and how these can be accessed, including their GP practice and, where they are eligible, the NHS programme. Explain the benefit of vaccination and the risks of COVID-19. Document any advice given and the decision reached. Inform or refer to the GP as appropriate. Where myocarditis or pericarditis followed a previous dose, refer for specialist advice.

Vaccines covered by this PGD

Confirm the presentation, licensed age range and dose volume against the current SPC before administration. Presentations and approved formulations change between seasons.

Vaccine	Age under this PGD	Dose and route	Notes
Comirnaty JN.1 30 micrograms/dose dispersion for injection (Pfizer BioNTech)	12 years and over	30 micrograms in 0.3 ml, intramuscular	Vaccine of choice under this PGD. mRNA vaccine. Confirm the seasonal formulation on the pack. Age-specific paediatric presentations exist but are outside this PGD.
Spikevax JN.1 0.1 mg/mL dispersion for injection (Moderna)	12 years and over	Single dose of 0.5 ml, intramuscular, confirm the strength and volume for the presentation supplied against the current SPC	Use where Comirnaty is unavailable. mRNA vaccine.
Nuvaxovid JN.1 dispersion for injection in pre-filled syringe (Novavax)	12 years and over	0.5 ml, intramuscular	Use where the mRNA vaccines are unavailable or unsuitable. Recombinant protein vaccine with Matrix-M adjuvant, contains polysorbate 80.

Details of the medicines

Legal category	POM
Black triangle	Check the current SPC and MHRA guidance for each product. Report all suspected adverse reactions via the Yellow Card scheme.
Off-label use	This PGD does not authorise off-label use. Administration must be within the licensed age, presentation and dose stated in the current SPC for the product supplied.
Storage	Store in accordance with the current SPC for the product and presentation supplied, including any thawing, dilution and in-use stability requirements for the mRNA vaccines. Do not freeze Nuvaxovid. Vaccine exposed to conditions outside the stated range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance.
Route/method of administration	Administer by intramuscular injection into the deltoid muscle of

	the upper arm. • Prepare in accordance with the SPC for the presentation supplied, including thawing and, where applicable, dilution and mixing. Inspect visually and do not administer if the appearance differs from that described in the SPC. • Where given with other vaccines, including influenza vaccine, use separate sites, preferably different limbs, or at least 2.5 cm apart in the same limb. Record the site of each.
Dose and frequency of administration	A single dose per campaign, as set out in the vaccines table above. Minimum interval of 3 months from the previous COVID-19 vaccine dose. This PGD does not cover primary courses in individuals who are unvaccinated and immunosuppressed, or any schedule requiring more than one dose in the season; refer these individuals.
Quantity to be administered and/or supplied	One dose of the selected vaccine, at the volume stated in the vaccines table.
Maximum or minimum treatment period	One dose for the 2026/27 season. This PGD expires at the end of the 2026/27 season and must be reviewed and reissued, including confirmation of the approved seasonal formulation.
Disposal	Dispose of used syringes, needles, vials and any discharged vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Drug interactions	May be given at the same time as influenza vaccine and other vaccines at a separate site. The immune response may be reduced in individuals receiving immunosuppressive treatment; this is not a reason to withhold vaccination. Refer to the current SPC for each product.
Adverse effects	Very common and common: injection site pain, swelling and redness, fatigue, headache, myalgia, arthralgia, chills, fever, nausea and lymphadenopathy. Rare: hypersensitivity reactions including anaphylaxis. Very rare: myocarditis and pericarditis, reported mainly after mRNA vaccines. Refer to the current SPC and PIL for each product for the full list.
Yellow card reporting	Report suspected adverse reactions to the MHRA via the Yellow Card scheme at https://yellowcard.mhra.gov.uk . Document any adverse reaction in the individual's record and inform their GP.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP with whom the individual is registered • name of immuniser • name and brand of vaccine • date of administration • dose, form and route of administration • quantity administered • batch number and expiry date • anatomical site of vaccination • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken • that the vaccine was administered via PGD • the product and seasonal formulation administered • the date of the previous COVID-19 vaccine dose, where known Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's

	GP should be informed. - Adults aged 18 years and over: keep records for 8 years - Children aged under 18 years: keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Offer the marketing authorisation holder's patient information leaflet (PIL) for the product administered, and a written record of the vaccine given with the date, brand, formulation and batch number.
Follow-up advice to be given to patient or carer	Inform the individual of possible side effects and their management, and advise them to seek medical advice in the event of an adverse reaction and to report it via the Yellow Card scheme. Advise them to seek urgent medical attention if they develop chest pain, shortness of breath, palpitations or a fluttering heartbeat after vaccination. Explain that protection develops over about 1 to 2 weeks, wanes over time, and that vaccination does not remove the need to seek advice if they become unwell. Vaccines do not provide 100% protection.

Key references

- Immunisation Against Infectious Disease (the Green Book), COVID-19 chapter (chapter 14a) - JCVI statement on COVID-19 vaccination in autumn 2026 and spring 2027, 16 July 2025, and the government response accepting that advice - Regulatory recommendations on the antigenic composition of COVID-19 vaccines for the 2026 to 2027 season - Summary of Product Characteristics for Comirnaty, Spikevax and Nuvaxovid, current versions - UKHSA Vaccine Incident Guidance - NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions - British National Formulary (BNF)

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
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25/8/26

31/1/27

01/09/26

31/1/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		23/8/26
Chris Pilkington	Pharmacist GPhC: 2046322		23/8/26

Change history

Version number	Change details	Date
001	Development and issue of new PGD, Nuvaxovid and Comirnaty, 12 years and over	25th August 2025
002	Reissued for the 2026/27 season. Spikevax added alongside Comirnaty and Nuvaxovid, with Comirnaty named as the vaccine of choice and the others as alternatives where stock is unavailable. Superseded KP.2 and LP.8.1 variant wording replaced with the 2026/27 seasonal formulation and a requirement to confirm the formulation against the pack. Eligibility, minimum 3 month interval, myocarditis and pericarditis exclusion and counselling, pregnancy, immunosuppression and NHS entitlement wording added. Age range unchanged at 12 years and over.	30th July 2026